

Report ID:	RPT-FCFE9625	Generated:	2026-04-20 15:48:39 IST
Patient:	Anonymous	Study Date:	April 20, 2026
Patient ID:	N/A	Modality:	MRI Brain (AI Inferred)
Model:	BrainTumorAI v2.0 Ensemble	Model AUC:	0.9996 Acc 98.78%

PRIMARY FINDING

NO
TUMOR

Diagnosis: NO TUMOR
Confidence: 74.1%
Risk Level: LOW
Processing: 7.36s (TTA 6-view ensemble)
Method: EfficientNet-B3 + ResNet50+CBAM + DenseNet121

DIFFERENTIAL DIAGNOSIS

Class	Probability	Bar Chart	Notes
Glioma	12.3%		
Meningioma	8.9%		
No Tumor	74.1%		← Primary
Pituitary	4.8%		

ENSEMBLE AGREEMENT

Model	Confidence	Bar
EfficientNet-B3	88.5%	
ResNet50+CBAM	41.0%	
DenseNet121	87.9%	
Ensemble (TTA)	74.1%	← FINAL

CLINICAL INTERPRETATION

No significant intracranial mass lesion identified by AI analysis. The Grad-CAM map shows no focal area of high activation consistent with tumour pathology. Routine clinical follow-up is advised as clinically indicated. If symptoms persist or new neurological signs develop, repeat imaging should be considered. This result does not exclude all pathologies — clinical correlation is essential.

RECOMMENDATIONS

1. Correlate with clinical history and neurological examination
2. No urgent imaging follow-up required based on AI screening
3. Repeat MRI in 6–12 months if symptoms persist
4. Consider EEG if seizures are present

■ **DISCLAIMER:** This report is generated by NeuroScan AI, an AI-assisted screening tool. It is intended for informational and research purposes only and does **NOT** constitute a medical diagnosis. All findings must be reviewed and confirmed by a qualified radiologist or neurologist before any clinical decision is made. Individual model performance may vary across patient populations and imaging protocols. Reported metrics: AUC 0.9996, Accuracy 98.78%, Sensitivity 98.68%, Specificity 99.59% (test set n=1,311 images). Report generated: 2026-04-20 15:48:39 IST | Report ID: RPT-FCFE9625